

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

September 6, 2023

MEMORANDUM FOR:

(b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

THROUGH:

(b)(6),(b)(7)(C),(b)(7)(F) MD
Deputy Assistant Director of Clinical Services/Medical Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

(b)(6),(b)(7)(C),(b)(7)(F) MS
Chief of Staff
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

FROM:

CAPT (b)(6),(b)(7)(C),(b)(7)(F), PA-C
Chief, Investigation Unit
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

SUBJECT:

Report of Findings – Mortality Review
James Thomas HILL, (b)(6),(b)(7)(C)

Executive Summary:

On August 6, 2020, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE noncitizen James Thomas HILL, (b)(6),(b)(7)(C). ICE assigned Mr. HILL, a 72-year-old Canadian male, to Immigration Centers of America-Farmville (ICAF) in Farmville, Virginia (VA), from April 15 to August 5, 2020, the date of his death. Mr. HILL's death certificate showed his cause of death as Coronavirus 2019 (COVID-19) viral infection.

On July 1, 1982, the former Immigration and Naturalization Service admitted Mr. HILL to the United States in Buffalo, New York, as a lawful permanent resident.

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On January 31, 2006, the Federal Bureau of Investigation in Shreveport, Louisiana (LA), arrested Mr. HILL for violation of 18 USC 1347 and 21 USC 841(a)(1), health care fraud, and distributing a controlled substance.

On March 21, 2007, the U.S. District Court for the Western District of Louisiana in Shreveport, LA, convicted Mr. HILL of health care fraud and distributing a controlled substance and sentenced him 320 months of incarceration.

On June 14, 2017, ICE Enforcement and Removal Operations (ERO) New Orleans lodged an immigration detainer - Notice of Action, Form I-247A, with the Adams County Detention Center in Natchez, Mississippi.

On March 3, 2020, ERO Washington encountered Mr. HILL at the Rivers Correctional Institute (RCI) in Winton, North Carolina, and confirmed the lodged I-247A remained valid.

On April 15, 2020, RCI released Mr. HILL to ERO Washington. ERO Washington served Mr. HILL a Notice to Appear, Form I-862, charging removability pursuant to Sections 237(a)(2)(A)(iii) and 237(a)(2)(B)(i) of the Immigration and Nationality Act, as an alien convicted of an aggravated felony and an alien with a controlled substance conviction.

On the same day, ICE booked Mr. HILL into ICAF. A registered nurse (RN) completed Mr. HILL's health assessment, and he reported a history of significant hearing loss, use of bilateral hearing aids, tonsillectomy, left (L) inguinal and L knee surgery/hospitalizations, but denied any chronic conditions. The RN documented normal vital signs (VS), and a normal health assessment. The RN cleared Mr. HILL for housing in general population (GP) and assigned him to the medical housing unit (MHU) for 14-days, in accordance with the facility's COVID-19 guidelines. Mr. HILL's chest x-ray showed negative results for tuberculosis (TB).

On May 12, 2020, an immigration judge in Arlington, VA, ordered Mr. HILL's removal from the United States to Canada. Mr. HILL waived his right to appeal.

On July 3 and 10, 2020, Mr. HILL required hospitalization for shortness of breath (SOB), body aches, headaches, and generalized weakness and low oxygen saturation [(SpO2): a measurement of the amount of oxygen in the blood] of 80 – 83 percent [(%); normal 95 – 100%], respectively.

On August 5, 2020, at approximately 10:56 p.m. (EDT), Centra Lynchburg General Hospital (CLGH) removed Mr. HILL's ventilator support per his family's request, and at 11:42 p.m., a CLGH attending physician pronounced Mr. HILL deceased.

Mortality Finding:

As a result of Mr. HILL's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his

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death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures, or practices are warranted; and identify issues that require further study.

A mortality review determined the care ICAF provided Mr. HILL deviated beyond safe limits but did not directly contribute to his death. Specifically, the mortality review participant determined the following: 1) missing or lack of documentation, which included complete VS, oxygen saturation levels, incident reporting, and completion of segregation logs; and 2) Mr. HILL did not receive timely sick call evaluation or a referral to a higher-level provider when his VS and oxygen saturation levels went below the MD's set parameters.

On September 5, 2023, Fact Finder (FF) CAPT (b)(6),(b)(7)(C),(b)(7)(F) discussed the events surrounding Mr. HILL's death with Deputy Assistant Director of Clinical Services/Medical Director, Dr. (b)(6),(b)(7)(C),(b)(7)(F) physician reviewer.

The following summary of events was based on: 1) Mr. HILL's ICAF medical records; 2) Mr. HILL's incident and notification reports; 3) (b)(7)(E) and (b)(7)(E) database records; 4) Mr. HILL's detention file; 5) Mr. HILL's death certificate; 6) video surveillance footage; 7) a remote review and staff interviews conducted by FF CAPT (b)(6),(b)(7)(C),(b)(7)(F) from September 29 to October 1, 2020; and 8) applicable ICAF policies and procedures, and ICE Detention Standards.

Summary of Events

On April 15, 2020, ICE booked Mr. HILL into ICAF. An RN completed Mr. HILL's health assessment, and he reported a history of significant hearing loss, use of bilateral hearing aids, tonsillectomy, L inguinal and L knee surgery/hospitalizations, but denied any chronic conditions. The RN documented normal VS, and a normal health assessment. The RN cleared Mr. HILL for housing in GP and assigned him to the MHU for fourteen days, in accordance with the facility's COVID-19 guidelines. Mr. HILL's chest x-ray showed negative results for TB.

On April 16 and 28, 2020, Mr. HILL received COVID-19 symptom screening, twice daily for fourteen days. During this period, Mr. HILL denied COVID related symptoms (i.e., fever, body aches, SOB, severe headache, chills, sore throat, new frequent cough, or nausea, vomiting, or diarrhea).

On April 29, 2020, a medical doctor (MD) discontinued Mr. HILL's COVID-19 symptoms checks and cleared him to go to GP but not to the dorms used to house patients with COVID-19 symptoms (b)(7)(E).

On May 2, 2020, Mr. HILL submitted an urgent sick call request to discuss his concerns of developing COVID-19, requested an alternate housing assignment, other than GP, due to his

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knowledge of noncitizens with COVID-19 in his dorm. A licensed practical nurse (LPN) referred Mr. HILL to an MD for evaluation.

On May 4, 2020, the MD evaluated Mr. HILL regarding his housing concerns and risk(s) for COVID-19 exposure. The MD documented Mr. HILL status as no acute distress, recognized his increased risk of developing COVID-19 complications if he contracted the virus, and plan to speak with the facility's operation team to identify a more conducive placement for him in the dorm. If not an option, the MD advised Mr. HILL to request protective custody for social isolation.

On May 14, 2020, Mr. HILL submitted a second sick call request to discuss his housing concerns and risk (s) for COVID-19 exposure. As a result, he requested placement in the medical department, or release from the facility.

On May 19, 2020, the MD evaluated Mr. HILL for his concerns related to COVID-19 exposure and his continued housing assignment in GP. Mr. HILL reported he would leave "soon in days," and no longer felt the need for an alternate housing assignment. The MD educated Mr. HILL on increased hand washing and social separation.

On May 26, 2020, Mr. HILL submitted a sick call request for frontal headaches worst at night, myalgia (muscle pain), fever, persistent nasal congestion, and dry cough; however, the medical staff did not receive the request until June 3, 2020, and the nursing staff did not evaluate Mr. HILL until June 4, 2020. Later in the day, security staff transferred Mr. HILL to dorm 7, bunk 80.

On May 31, 2020, Mr. HILL submitted a follow-up sick call request for his persistent frontal headache, nasal congestion, myalgia, fever, and dry cough for two days, worst at night. Later in the evening, an LPN checked Mr. HILL's VS and documented his blood pressure (BP) of 128/82 millimeters of mercury [(mmHg; normal range is more than 90/60 and less than 120/80 mmHg] and temperature (T) of 98.8 degrees (°) Fahrenheit (F) (normal range 97.8 – 99.1°F). The LPN referred Mr. HILL for a sick call evaluation.

On June 1, 2020, an RN evaluated Mr. HILL, documented normal VS, no acute distress, and scheduled Mr. HILL a routine medical evaluation. The RN referred Mr. HILL for a routine follow-up within seven days and did not issue any over-the-counter medications.

On June 3, 2020, an RN triaged Mr. HILL's sick call request from May 26, 2020, documented normal VS, and referred him for a sick call evaluation.

On June 4, 2020, an LPN documented Mr. HILL refused his sick call evaluation, because of his self-report of improvement of symptoms.

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On June 16, 2020, Mr. HILL submitted a sick call request for triple antibiotic cream for left ear pain for seven days, due to wearing earplugs.

On June 17, 2020, an RN reviewed Mr. HILL's sick call request from the previous day, documented normal VS, and referred him for a sick call evaluation.

On June 18, 2020, an LPN evaluated Mr. HILL for irritation and mild L ear pain, documented normal VS, and the MD gave a verbal order to start triple antibiotic ointment for five days.

On July 1, 2020, during a use-of-force incident, a custody officer discharged oleoresin capsicum (OC) pepper spray into Mr. HILL's dorm. Mr. HILL did not receive a medical evaluation after the use-of-force incident.

On July 2, 2020, Mr. HILL submitted a sick call request for a low-grade fever, nasal congestion, chest tightness, wheezes, body aches, and a productive cough status post his exposure to OC spray on the previous day. An unknown provider completed Mr. HILL's COVID-19 test and sent the sample to an off-site laboratory for processing; however, the laboratory did not complete the test needed to rule out COVID-19, due to an influx of tests requiring processing.

On July 3, 2020, Mr. HILL's dorm officer contacted the medical department, reported Mr. HILL's complaints of SOB, body aches, headaches, and generalized weakness. An LPN evaluated Mr. HILL, documented normal VS, except for his temperature of 99.1°F, and his diminished breath sounds bilaterally and fine crackles, upon auscultation. The MD gave a verbal order to transfer Mr. HILL to Centra Southside Community Hospital (CSCH), for further evaluation.

On July 4, 2020, CSCH diagnosed Mr. HILL with bronchospasm (constriction of the muscles in the walls of the bronchioles), SOB, suspected COVID-19 viral infection, thrombocytopenia (low platelet level), and discharged Mr. HILL back to ICAF with a prescription order for a steroid and albuterol inhaler to treat his bronchospasm, and placement into respiratory isolation, until his COVID-19 results returned. Later in the day, the MD gave a verbal order to allow Mr. HILL to return to his dorm with VS checks, twice daily, pending the MD evaluation.

- At approximately, 5:45 p.m., an LPN evaluated Mr. HILL in his dorm, noted him completely covered with sheets. Mr. HILL refused to have his complete VS performed. The LPN documented Mr. HILL's temperature of 101.8°F and plan to notify the health services administrator.
- At approximately 8:00 p.m., the MD gave an LPN a verbal order to transfer Mr. HILL into the MHU, for placement into a respiratory/contact isolation room, and to perform VS checks oxygen saturation checks every four hours.

Between July 5 and 9, 2020, Mr. HILL's symptoms and VS fluctuated, and he reported having multiple symptoms, including not feeling well, nosebleeds during the night, weakness, fever,

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SOB, diarrhea, and chest tightness, and presented with an episode of unsteadiness while standing. Mr. HILL's COVID-19 test results completed in CSCH remained pending.

On July 7, 2020, the MD ordered Mr. HILL benzonatate (cough suppressant), a drink containing electrolytes (Gatorade), and acetaminophen (pain and anti-fever) medications, baseline laboratory studies and a chest x-ray that showed negative result for TB the following day, and documented Mr. HILL's symptoms may be due to COVID-19 or related to his previous exposure to the OC spray.

On July 10, 2020, Mr. HILL reported feeling "weak." The MD evaluated Mr. HILL, documented his report of having mild hallucinations. The MD documented his low oxygen saturation level ranging between 85 – 90%, "symptoms concerning for COVID-19, and ordered Mr. HILL's transfer to the hospital if his oxygen saturation levels remained below 90% and to notify the MD.

- At Approximately 7:00 p.m., an RN documented Mr. HILL's VS as normal, but his oxygen saturation decreased to 88%. The RN did not notify the MD.
- At approximately 9:45 p.m., ICAF transferred Mr. HILL to CSCH, due to low oxygen saturation of 80 – 83%, where he required subsequent admission into the hospital for COVID-19 infection.

On July 11, 2020, CSCH transferred Mr. HILL to CLGH in Lynchburg, VA, for higher-level of care, due to low oxygen saturation levels, positive test for COVID-19, and worsening respiratory symptoms. Mr. HILL tested positive for COVID-19.

Between July 12 and 29, 2020, Mr. HILL's symptoms worsened, and he developed a left apical pneumothorax (a condition where air leaks into the space between the lungs and chest wall) and extensive subcutaneous (under the skin) emphysema (a lung condition that causes shortness of breath), which required admission into the intensive care unit (ICU), intubation, chest tube placement, and ventilation assistance.

On July 15, 2020, Mr. HILL's ICAF's COVID -19 test results, dated July 4, 2020, showed "test not performed, due to specimen handling error."

On July 29, 2020, CLGH medical staff notified ERO Washington that Mr. HILL's prognosis remained poor, but stable. A CLGH MD spoke to Mr. HILL's family (niece and nephew), who authorized a "Do Not Resuscitate" order.

On August 1, 2020, CLGH medical staff notified ERO Washington that Mr. HILL had multi-organ system failure, with a very poor prognosis.

On August 5, 2020, at approximately 10:56 p.m., CLGH removed Mr. HILL's ventilator support per his family's request.

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- At approximately 11:42 p.m., a CLGH attending physician pronounced Mr. HILL deceased.

On August 11, 2020, the Commonwealth of Virginia issued Mr. HILL's death certificate, which identified Mr. HILL's cause of death as COVID-19 viral infection.

ICE detention standards used for this review: Performance-Based National Detention Standards (PBNDS), 2011. The following are identified program strengths, weaknesses, and recommendations found during this review.

Strengths

Strengths identified during this investigation were: 1) ICAF medical staff provided requested information timely and remained receptive to feedback; and 2) Administrators demonstrated proactivity and completed an internal review of the events surrounding Mr. HILL's death in efforts to implement immediate corrective actions.

Weaknesses

1. Patient assessments and documentation.

- On July 3 and 5, 2020, nursing notes did not include all essential elements of a health record, which include: addressing significant findings, treatments, and/or dispositions.
- Between July 4 and 10, 2020, an MD ordered VS every four hours and/or to receive notification if Mr. HILL's oxygen saturation level decreased below 90%; however, the nursing staff did not follow the order each time, as directed.
- On July 1, 2020, ICAF conducted a use-of-force in Mr. HILL's dorm and deployed OC; however, the medical staff did not evaluate Mr. HILL after the incident and did not complete an incident report to document the incident into his electronic health record.
- On May 14 and 26, 2020, Mr. HILL submitted a sick call request, but the medical staff did not triage and/or evaluate Mr. HILL timely.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section BB. *Medical Records*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section M. *Comprehensive Health Assessment*.
- NCCHC 2018 standards: J-A-08, *Health Records*.
- NCCHC 2018 standards: J-E-09, *Continuity, Coordination, and Quality of Care During Incarceration*.

Recommendations

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- Forward these findings to the IHSC Deputy Assistant Director of Health Care Compliance (DAD of HCC) for awareness.
- Since this mortality occurred on October 1, 2019, the IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, OMD administrator and health authority for awareness and to implement any action(s) deemed appropriate.

End of report.